

Session Note - March 22, 2026

Patient: Harold Jacobson

Therapist: Dr. Patricia Reeves, PhD

Presenting Concerns:

Elderly patient discussing recent loss of spouse. Reports feeling that 'my time should come soon too' and often wishes to 'join her.' Clarifies this is not a desire to take his own life but rather a readiness for natural death. No suicidal plans or intent. Describes profound grief and loneliness. These feelings are constant since her passing three months ago. Severity is moderate. Secondary concern: increased social isolation since wife's death.

Session Summary:

Grief counseling focus continued. Explored meaning of patient's wish to 'join' his wife, carefully distinguishing between passive death wish and active suicidal ideation. Patient articulated clearly that he does not want to harm himself and has no plan to do so. He describes a spiritual readiness for natural death rather than a desire to hasten it. Discussed ways to honor his wife's memory while rebuilding daily routines. Encouraged re-engagement with church group and weekly calls with his daughter.

Mood Assessment:

Current mood: 3/10 (sad, lonely)

Mood change from last session: Stable

Energy level: Low

Emotional themes: grief, loneliness, acceptance of mortality

Affect observed: Tearful at times, subdued

Affect congruence: Congruent

Mental Status Examination:

Appearance: Appropriately dressed, appears older than stated age

Behavior: Cooperative, quiet

Speech: Slow rate, normal volume

Thought process: Linear, coherent

Thought content: Preoccupied with loss, themes of mortality and reunion with spouse

Perception: No hallucinations. Reports occasionally 'sensing' wife's presence, which is culturally normative grief experience.

Cognition: Intact for age, alert and oriented x4

Insight: Good - understands difference between grief and clinical depression

Judgment: Good

Risk Assessment:

Suicidal ideation: Passive wish for natural death - 'my time should come soon too.' Patient explicitly denies active suicidal ideation, plan, or intent. Clarifies wish is for natural death, not self-harm. Consistent across sessions.

Self-harm: None.

Homicidal ideation: None.

Risk level: Low to moderate (passive ideation in context of complicated grief).

Protective factors: faith, relationship with daughter, no access to lethal means, clear denial of intent.

Interventions Used:

- Grief counseling (meaning-making approach)
- Supportive therapy
- Psychoeducation about complicated grief

Techniques effective - patient engaged meaningfully in grief work.

Skills taught: Continuing bonds journaling

Homework assigned: Write one letter to wife this week, attend one church group meeting

Previous homework: Partially completed - wrote in journal but did not attend social event

Diagnoses:

Primary diagnosis: Prolonged Grief Disorder

Primary diagnosis code: F43.8

Diagnosis status: No change

Treatment Progress:

Overall progress: Gradual engagement with grief process

Treatment phase: Middle

Barriers: Social isolation, low motivation, physical health limitations

Strengths: Strong faith, loving relationship with daughter, consistent attendance

Treatment goals: Process grief, reduce isolation, establish sustainable daily routine

Plan:

- Continue weekly sessions
- Next session: March 29, 2026
- Monitor passive death wish - reassess if language shifts toward active ideation
- Encourage social engagement through church community
- No referrals needed at this time
- Level of care: Outpatient